

HIGHLIGHTS OF PRESCRIBING INFORMATION: IBUPROFEN TABLETS

WARNING: RISK OF SERIOUS CARDIOVASCULAR AND GASTROINTESTINAL EVENTS

Cardiovascular Thrombotic Events:

- Nonsteroidal anti-inflammatory drugs (NSAIDs) cause an increased risk of serious cardiovascular thrombotic events, including myocardial infarction and stroke, which can be fatal. This risk may occur early in treatment and may increase with duration of use.
- Ibuprofen is contraindicated in the setting of coronary artery bypass graft (CABG) surgery.

Gastrointestinal Bleeding, Ulceration, and Perforation:

- NSAIDs cause an increased risk of serious gastrointestinal (GI) adverse events including bleeding, ulceration, and perforation of the stomach or intestines, which can be fatal. These events can occur at any time during use and without warning symptoms. Elderly patients and patients with prior history of peptic ulcer disease and/or GI bleeding are at greater risk for serious GI events.

1 INDICATIONS AND USAGE

Ibuprofen is indicated for:

- Relief of signs and symptoms of rheumatoid arthritis and osteoarthritis.
- Relief of mild to moderate pain (headache, dental pain, musculoskeletal pain, dysmenorrhea).
- Reduction of fever.

2 DOSAGE AND ADMINISTRATION

- Use the lowest effective dosage for the shortest duration consistent with individual patient treatment goals.
- Rheumatoid Arthritis and Osteoarthritis: 1200 mg to 3200 mg daily divided into 3 or 4 equal doses (e.g., 400 mg, 600 mg, or 800 mg three or four times daily). Do not exceed 3200 mg per day.
- Mild to Moderate Pain: 400 mg every 4 to 6 hours as necessary. Over-the-counter maximum daily dose is 1200 mg/day unless directed by a doctor.

4 CONTRAINDICATIONS

- Known hypersensitivity to ibuprofen or other NSAIDs (history of asthma, urticaria, or allergic-type reactions after taking aspirin or other NSAIDs).
- In the setting of coronary artery bypass graft (CABG) surgery.

5 WARNINGS AND PRECAUTIONS

- Cardiovascular Thrombotic Events: See Boxed Warning. Avoid use in patients with recent MI unless benefits outweigh risks.
- Gastrointestinal Bleeding and Ulceration: See Boxed Warning.
- Hepatotoxicity: Borderline elevations of liver tests may occur in up to 15% of patients taking NSAIDs.
- Hypertension: NSAIDs can lead to new onset or worsening of preexisting hypertension.
- Heart Failure and Edema: NSAIDs may blunt cardiovascular response to loop diuretics and worsen heart failure.
- Renal Toxicity and Hyperkalemia: Long-term administration can result in renal papillary necrosis and other renal injury. Monitor renal function in patients with renal impairment, heart failure, liver dysfunction, or those taking ACE inhibitors.